

LIBBY CASE REPORT

demo-nsc lc-egfr-l858r-post-osi-d3m0

Multi-agent decision support — clinician track

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Libby is an experimental decision-support tool. The recommendations in this report have not been reviewed by a clinician treating this patient. Do not act on this report without consulting a qualified oncologist.

Executive summary

Case:demo-nsclc-egfr-l858r-post-osi-d3m0**Question:**EGFR L858R-mutant stage IV lung adenocarcinoma with on-target MET amplification (GCN 8.2) after 22 months of first-line osimertinib. T790M is absent. What's the next line, given a working artist who needs her hands and prefers trials and oral therapy? **Evidence base:**4 trials, 5 clinical-evidence rows, 4 preclinical rows. Three ranked interventions. Board agreement scores span +0.8 to -0.4, so rank 1 carries near-consensus and rank 2 carries an active veto.

What this report covers

The PI's synthesis of the post-osimertinib decision when MET amplification is the dominant resistance driver. There are no scenario branches in this case; the three ranked options are unconditional, and the gap between rank 1 and rank 2 is large enough that the report reads as one main recommendation plus two transparency entries.

Top-line findings

- MET amplification at GCN 8.2 by FISH, with T790M absent, points cleanly at a MET-directed combination. Savolitinib plus osimertinib is the option that fits both the biology and the patient's stated preferences.
- The board endorsed rank 1 four-to-one. The consensusite flagged that savo-osi is not yet inside NCCN; the resolution is to enroll on SAFFRON or SACHI rather than treat off-trial, which the patient already prefers.
- The conservative vetoed rank 2 (amivantamab + lazertinib) on severe-neuropathy and IV-burden grounds. Critic and advocate also dissented. Two personas endorsed it on guideline alignment and effect size, so it stays on the table as considered_with_caveats rather than being dropped — but override of the veto is not recommended unless rank 1 fails.
- Rank 3 (patritumab deruxtecan) triggers the patient's alopecia veto outright. Only the risktaker advanced it. It's listed for transparency, not as a live option.

Recommendation summary

1. **Savolitinib + osimertinib (preferably on SAFFRON / SACHI)**— recommended. ORR ~30% in the MET-amp post-EGFR-TKI stratum (TATTON expansion, PMID 32679432). All-oral; clears every stated veto; trial-route resolves the guideline gap.
2. **Amivantamab + lazertinib**— considered with caveats. ORR 36% in the CHRYSALIS-2 post-osimertinib subset. Conservative veto holds on neuropathy and IV chair time, both of which matter unusually for a patient whose work depends on her hands.
3. **Patritumab deruxtecan (HER3-DXd)**— considered with caveats. Triggers the alopecia veto. Surfaced so the user can revisit it with her oncologist if she chooses to re-weight that veto.

What this report does *not* cover

Dose adjustments, monitoring schedules, sequencing across later lines, payer or access logistics, or institution-specific trial slot availability. Those belong to the treating team.

How to use this report

Each ranked option carries the board's agreement state (endorsed, dissent, veto by persona) and one to three anchor citations. The full per-trial extraction lives on the live case page; this PDF is the synthesis.

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Profile snapshot (scrubbed)

- **Primary site / histology:**lung adenocarcinoma
- **Stage:**IV
- **Performance status:**ECOG 1
- **Age band:**60-69
- **Biomarkers:**EGFR L858R (present); EGFR T790M (absent); MET amplification (GCN 8.2); PD-L1 TPS 10%
- **Prior therapy:**1L osimertinib, PR x 22 months
- **Targetable features:**EGFR L858R; MET amplification (likely on-target resistance)

Preferences

- **Efficacy/toxicity weight:**0.55 (slight efficacy lean)
- **Toxicity vetoes:**severe neuropathy, cardiotoxicity, alopecia
- **Modality:**oral preferred; no inpatient infusion; minimize IV chair time
- **Free text:**working artist — manual dexterity in hands matters more than typical
- **Trials preferred:**yes

Recommendations summary

Rank	Status	Intervention	Endorsed	Dissent	Veto
1	recommended	savolitinib + osimertinib (preferably on SAFFRON / SACHI)	risktaker, conservative, critic, advocate	—	—
2	considered with caveats	amivantamab + lazertinib	risktaker, consensusite	critic, advocate	conservative
3	considered with caveats	patritumab deruxtecan (HER3-DXd)	risktaker	—	—

See [Recommendations](#) for the full table with rationale and open questions.

Synthesis

The board converged on **savolitinib + osimertinib** as the rank-1 recommendation: four of five personas endorsed; the lone qualifier (consensusite) flagged a guideline-fit concern that resolves on trial enrollment, which is itself preferred by the patient. The combination's evidence anchors are TATTON expansion (PMID 32679432) and the SAFFRON / SACHI phase-3 program (NCT05261399). All stated preference axes — oral, no IV burden, no severe neuropathy / cardiotoxicity / alopecia — clear cleanly.

Amivantamab + lazertinib drew a toxicity veto from the conservative on neuropathy and IV-burden grounds, with concurring dissent from the advocate on preference grounds and the critic on evidence-quality grounds. Consensusite endorsed on NCCN 2A alignment; risktaker endorsed on effect-size grounds. Per PI rules a veto is never silently dropped — the option remains on the table marked `considered_with_caveats` so the patient and clinician can see what was considered. **Override of the conservative's veto is not recommended absent**

failure of the rank-1 option.

Patritumab deruxtecan was advanced only by the risktaker and triggers the alopecia veto outright; surfaced for transparency only.

Transparency artifacts

- [Trial table](#)— 4 trials in the dossier
- [Evidence list](#)— clinical + pre-clinical
- [Tumor-board transcript](#)— 5 positions, 20 cross-critiques
- [Recommendations table](#)— full ranked detail
- [Plain-language summary](#)— patient/caregiver track

Run log

Demo case authored May 2026 with synthetic data for smoke-testing the pipeline. No real patient information is represented.

Sources

PubMed (PMID):

- [32679432](#)
- [36720074](#)
- [37563559](#)

ClinicalTrials.gov (NCT):

- [NCT05261399](#)